

AFTER VISIT SUMMARY

Lisa A. English MRN: 055656 CEID: DCN-66SG-16HJ-T354

 Physical debility  6/19/2026 - 6/23/2026  Med/Surg Midtown

Instructions



This is a summary of your visit. Please, also refer to any additional handouts or educational material that may have been provided to you during your visit.

Contact your Health Care Provider if:

- You have new or worse signs and symptoms.
- Your prescribed medicine does not seem to be working.
- You have a fever, chills, or other cold or flu-like symptoms.
- You have questions or concerns about your condition or care.



Your medications have changed

 **START taking:**
benzonatate (TESSALON)
ondansetron (ZOFTRAN ODT)

 **CHANGE how you take:**
aspirin
predniSONE (DELTASONE)

 **STOP taking:**
amoxicillin-clavulanate 875-125 MG per tablet (AUGMENTIN)

Review your updated medication list below.

About Your Hospitalization

Hospital Information

You were discharged on: June 23, 2026
You last received care in the: Med/Surg Midtown
Unit phone number: 812-450-5100

Why you were hospitalized

Your primary diagnosis was: Physical Debility



Your Next Steps



- ☐ Pick up 24 medications from Omnicare of Henderson - Henderson, KY - 1900 Highway 41 North



- ☐ Read these attachments
 - Dyspnea (General Information) (English)
 - COPD (Chronic Obstructive Pulmonary Disease) (Discharge Care) (English)



JUN 24

TCM 10:00 AM

Jung Kang Smith
Deaconess Clinic Mary
520 Mary Street, Ste 340
Evansville IN 47710-1679
812-450-6700

Enter the Physician Center Building (Subway Building) located at 520 Mary Street. Take the elevator (to the left) to the 3rd floor. Exit the elevator and take a left. Suite 340 will be on your right.

From the attached parking garage: Enter the Physician Center Building from the second floor of the parking garage. You will enter the hospital on the second floor. Turn right after the restrooms and take the elevators to the 3rd floor. Exit the elevators and turn left. Suite 340 will be on your right.

You have more future appointments. Please review your full appointment list.

MyChart

View your After Visit Summary and more online at <https://www.viewmychart.com/mychart/>.

Your diagnoses also included: Copd, Very Severe (Hcc), Dependence On Non-Invasive Ventilation (Hcc), Chronic Respiratory Failure With Hypoxia and Hypercapnia (Hcc), DM Hyperosmolarity Type II, Uncontrolled (Hcc), Generalized Anxiety Disorder, Morbid Obesity Due to Excess Calories (Hcc)

Your Latest Vitals



Blood Pressure
146/80



BMI
30.88



Weight
179 lb 14.3 oz



Height
5' 4"



Temperature (Oral)
98 °F



Pulse
82



Respiration
18



Oxygen Saturation
96%



BSA
1.92 m²

Providers seen during hospitalization

Provider	Role	Specialty
Malik, Ahmad T, MD	Attending	Hospitalist
Mansour, Joseph A, MD	Attending	Hospitalist
Jones, William T, MD	Attending	Emergency Medicine

You are allergic to the following

Allergen	Reactions
Trazodone Hcl Violent dreams	Other/Unknown (See Comments)
Sulfa Antibiotics	Tachycardia
Probiotic Bloating, gas, abdominal pain, constipation	Other/Unknown (See Comments)

Immunizations Administered as of 6/23/2026

Never Reviewed

Name	Date
Covid-19 28 Day Interval Moderna	2/4/2021, 1/7/2021
Covid-19 Moderna Booster Half Dose	12/17/2021
Covid-19 Vaccine Immunization Unspecified	9/21/2025
Covid-19, Moderna, 12+, LNP-S, PF 50 mcg/0.5mL	9/24/2023
HEP A/HEP B	3/16/2024, 9/24/2023
INFLUENZA SPLIT VIRUS PRESERVA	9/29/2020, 1/16/2020, 12/4/2018, 11/3/2015, 1/5/2015, 1/17/2014

Name	Date
INFLUENZA, FLUBLOK, PF EGG-FREE	11/15/2025
Influenza Quadrivalent, Preservative Free	9/23/2023, 10/20/2022
Influenza, MDCK, trivalent, PF >=6 mos	11/3/2015, 1/5/2015
Influenza, Quadrivalent	9/29/2020, 1/16/2020
Influenza, Quadrivalent (CCIIV4)	12/4/2018
Pneumococcal Conj PEDS preferred PCV20	9/24/2023
Pneumococcal Polysaccharide PPV23	1/16/2020, 11/1/2011
RSV, Recombinant Adjuvant	11/15/2025
Tdap	9/24/2023, 8/12/2012
Zoster (Shingles)	10/8/2021
Zoster Vaccine Recombinant	11/26/2022, 9/13/2022



Activity Orders

Activity

- Activity: Ambulate
- Activity: As tolerated

Activity Instructions:

- Activity as tolerated



Diet Instructions

Advance diet as tolerated

Diet Instructions:

Regular Diet:

- A regular diet is a healthy meal plan that includes a variety of healthy foods from all the food groups.
- Follow this meal plan if you do not have any health conditions that require a special diet.
- A healthy meal plan is low in unhealthy fats, salt, and added sugar.
- Eat a variety of vegetables, fresh fruits, whole grains, protein foods, and low-fat dairy products.
- Limit vegetables with added fat, fruit with added sugar, carbohydrates high in fat and sugar, protein foods with added fat, high-fat dairy products, and unhealthy fats.



Additional Instructions

Demonstration/instruction given to patient, family, and/or significant other is as follows:

{AVS Instructions Given to Patient:304120250}

Follow Up Services Contacted at Discharge:

{IP AVS Follow Up Services:304120020}

Discharge Disposition:

- Skilled Nursing Facility

Age Friendly

Flowsheet Row	Most Recent Value
Therapy Recommendations	Pt would benefit from continued therapy at a facility level
Equipment Recommended	Rolling walker, Reacher, Sock aid, Long-handled sponge
Level of Care Recommendation	—
Recommendation	Pt would benefit from continued therapy at a facility level
Level of Care Recommendation	—
Nutrition Recommendations	—
Nutrition Precautions	—

What's Next

JUN 24	TCM with Jung Kang Smith Wednesday Jun 24, 2026 10:00 AM	Deaconess Clinic Mary 520 Mary Street, Ste 340 Evansville IN 47710-1679 812-450-6700
JUN 30	Wellness Update Medicare with Jung Kang Smith Tuesday Jun 30, 2026 3:30 PM	Deaconess Clinic Mary 520 Mary Street, Ste 340 Evansville IN 47710-1679 812-450-6700
JUL 9	Hospital Follow Up with Danielle McFadden, NP Thursday Jul 9, 2026 4:00 PM (Arrive by 3:45 PM) Reminder: Patients and Visitors will be required to wear a mask at all times while in the facility	Deaconess Pulmonary Critical Care Group Medical Office Building 3 4015 GATEWAY BLVD STE 2121 Newburgh IN 47630-8925 812-450-7720
JUL 17	Office Visit/Return with Dan Vardi Friday Jul 17, 2026 2:00 PM (Arrive by 1:45 PM)	Deaconess Pulmonary Critical Care 519 Harriet Street Evansville IN 47710-1715 812-450-7720
JUL 29	Office Visit/Return with Dan Vardi Wednesday Jul 29, 2026 2:40 PM (Arrive by 2:25 PM)	Deaconess Pulmonary Critical Care 519 Harriet Street Evansville IN 47710-1715 812-450-7720
AUG 19	Office Visit/Return with Mary Kasel, NP Wednesday Aug 19, 2026 11:30 AM (Arrive by 11:15 AM)	Deaconess Clinic Palliative Care Mary St 520 MARY ST STE 340 EVANSVILLE IN 47710-1679 812-450-3241
	Follow Up with Dan Vardi Wednesday Aug 19, 2026 1:40 PM (Arrive by 1:10 PM)	Deaconess Hospital Chronic Obstructive Pulm Disease 600 Mary Street Evansville IN 47747-0001 812-490-4704



Other instructions

May Substitute Generic Equivalent Drugs

Medication List

STOP taking these medications



amoxicillin-clavulanate 875-125 MG per tablet
Commonly known as: AUGMENTIN

TAKE these medications

	Morning	Around Noon	Evening	Bedtime	As Needed
Accu-Chek Guide Me w/Device Kit Indications: Diabetes Provide per pt preference or as covered by insurance Check blood glucose levels once a day as ordered per Provider. DXE11.9 Last time this was given: No last hospital dose found For diagnoses: Steroid-induced diabetes mellitus, initial encounter					Indications: Diabetes Provide per pt preference or as covered by insurance Check blood glucose levels once a day as ordered per Provider. DXE11.9
 Accu-Chek Guide Test strip Please check blood sugars once a day per orders Last time this was given: No last hospital dose found For diagnoses: Steroid-induced diabetes mellitus, initial encounter Generic drug: glucose blood VI test What changed: • how much to take • how to take this • when to take this					Please check blood sugars once a day per orders
 Accu-Chek Softclix Lancets Misc Indications: Diabetes Provide per pt preference or as covered by insurance Check blood glucose levels once a day as ordered per Provider. DXE11.9 Last time this was given: No last hospital dose found For diagnoses: Steroid-induced diabetes mellitus, initial encounter What changed: • how much to take • how to take this • when to take this					Indications: Diabetes Provide per pt preference or as covered by insurance Check blood glucose levels once a day as ordered per Provider. DXE11.9

	Morning	Around Noon	Evening	Bedtime	As Needed
acetaminophen 500 MG tablet Commonly known as: TYLENOL Take 1 tablet (500 mg) by mouth every 6 (six) hours if needed for Pain Last time this was given: No last hospital dose found					 1 tablet
* albuterol 108 (90 Base) MCG/ACT inhaler Inhale 2 puffs by mouth every 6 (six) hours if needed for Wheezing or Shortness of Breath Indications: Chronic Obstructive Lung Disease Last time this was given: No last hospital dose found For diagnoses: COPD, very severe (HCC), Dyspnea on exertion					 2 puffs
* albuterol (2.5 MG/3ML) 0.083% nebulizer solution Commonly known as: PROVENTIL Take 1 vial (2.5 mg) by nebulization every 6 (six) hours if needed for Wheezing or Shortness of Breath Indications: Reversible Disease of Blockage in Breathing Passages, Spasm of Lung Air Passages Last time this was given: No last hospital dose found Doctor's comments: File to medicare part B: J44.9 For diagnoses: COPD, very severe, COPD, frequent exacerbations					 1 vial
 CHANGE aspirin 81 MG chewable tablet Take 2 tablets (162 mg) by mouth daily Do not start before June 26, 2026. Last time this was given: 162 mg on June 23, 2026 7:35 AM Start taking on: June 26, 2026 What changed: These instructions start on June 26, 2026. If you are unsure what to do until then, ask your doctor or other care provider.	 Wait until Jun 26 2 tablets				
atorvaSTATin 20 MG tablet Commonly known as: LIPITOR Take 1 tablet (20 mg) by mouth daily Last time this was given: No last hospital dose found For diagnoses: Mixed hyperlipidemia	 1 tablet				

	Morning	Around Noon	Evening	Bedtime	As Needed
 benzonatate 100 MG capsule Commonly known as: TESSALON Take 1 capsule (100 mg) by mouth 3 times daily as needed for Cough Indications: Cough Last time this was given: No last hospital dose found					 1 capsule
budesonide 0.5 MG/2ML nebulizer suspension Commonly known as: PULMICORT Take 2 mL (500 mcg) by nebulization 2 times daily Last time this was given: 500 mcg on June 23, 2026 7:52 AM For diagnoses: COPD, very severe	 2 mL		 2 mL		
busPIRone 5 MG tablet Commonly known as: BUSPAR Take 1 tablet (5 mg) by mouth 2 times daily Last time this was given: 5 mg on June 23, 2026 7:35 AM	 1 tablet		 1 tablet		
DSS 100 MG Caps Take 100 mg by mouth 2 times daily for 30 days Last time this was given: 100 mg on June 23, 2026 7:35 AM	 100 mg		 100 mg		
Dupixent 300 MG/2ML pen Inject 2 mL (300 mg) into the skin every 14 days for 30 days Last time this was given: No last hospital dose found For diagnoses: Steroid dependent on Dupixent Generic drug: dupilumab	Inject 2 mL (300 mg) into the skin every 14 days for 30 days				
ferrous sulfate 325 (65 Fe) MG tablet Take 1 tablet (325 mg) by mouth daily (with breakfast) Last time this was given: No last hospital dose found	 1 tablet				
FLUoxetine 20 MG capsule Commonly known as: PROZAC Take 1 capsule (20 mg) by mouth daily Last time this was given: 20 mg on June 23, 2026 7:36 AM	 1 capsule				
furosemide 20 MG tablet Commonly known as: LASIX Take 1 tablet (20 mg) by mouth daily Last time this was given: 20 mg on June 23, 2026 7:34 AM For diagnoses: Edema, unspecified type	 1 tablet				

	Morning	Around Noon	Evening	Bedtime	As Needed
 GNP Alcohol Swabs 70 % Pads Indications: Diabetes Provide per pt preference or as covered by insurance. For use to monitor blood glucose levels as ordered per Provider. DXE11.9 Last time this was given: No last hospital dose found For diagnoses: Steroid-induced diabetes mellitus, initial encounter What changed: • how much to take • how to take this • when to take this	Indications: Diabetes Provide per pt preference or as covered by insurance. For use to monitor blood glucose levels as ordered per Provider. DXE11.9				
guaiFENesin 600 MG SR tablet Commonly known as: Mucinex Take 2 tablets (1,200 mg) by mouth every 4 hours as needed for Congestion Last time this was given: Ask your nurse or doctor					 2 tablets
ipratropium-albuterol 0.5-2.5 (3) MG/3ML nebulizer solution Commonly known as: DUO-NEB Take 3 mL by nebulization 4 times daily Last time this was given: 3 mL on June 23, 2026 1:23 PM For diagnoses: COPD exacerbation (HCC)	Take 3 mL by nebulization 4 times daily				
lisinopril 20 MG tablet Commonly known as: ZESTRIL Take 1 tablet (20 mg) by mouth daily Last time this was given: 20 mg on June 23, 2026 7:36 AM	 1 tablet				
metFORMIN 500 MG tablet Commonly known as: GLUCOPHAGE Take 1 tablet (500 mg) by mouth 2 times daily Last time this was given: No last hospital dose found	 1 tablet		 1 tablet		
metoprolol succinate 50 MG XL tablet Commonly known as: TOPROL XL Take 1 tablet (50 mg) by mouth 2 times daily Last time this was given: 50 mg on June 23, 2026 7:34 AM	 1 tablet		 1 tablet		
morphine 10 MG/5ML solution Take 2.5 mL (5 mg) by mouth every 6 (six) hours if needed for Pain Indications: Pain Last time this was given: 0.5 mg on June 21, 2026 4:12 AM For diagnoses: Dyspnea, unspecified type					 2.5 mL

	Morning	Around Noon	Evening	Bedtime	As Needed
omeprazole 40 MG capsule Commonly known as: PRILLOSEC Take 1 capsule (40 mg) by mouth 1 (one) time each day for 30 days Indications: Gastroesophageal Reflux Disease Last time this was given: No last hospital dose found For diagnoses: Gastroesophageal reflux disease, unspecified whether esophagitis present			 1 capsule		
 ondansetron 4 MG disintegrating tablet Commonly known as: ZOFTRAN ODT Take 1 tablet (4 mg) by mouth every 8 (eight) hours if needed for Nausea Indications: Nausea and Vomiting Last time this was given: Ask your nurse or doctor Additional Information: Next dose available is 1749					 1 tablet
Ozempic (0.25 or 0.5 MG/DOSE) 2 MG/3ML injection Inject 0.25 mg into the skin once a week Indications: Type 2 Diabetes Last time this was given: No last hospital dose found For diagnoses: DM hyperosmolarity type II, uncontrolled (HCC) Generic drug: semaglutide	Inject 0.25 mg into the skin once a week Indications: Type 2 Diabetes				
 predniSONE 20 MG tablet Commonly known as: DELTASONE Take 1 tablet (20 mg) by mouth daily for 5 days, THEN 0.5 tablets (10 mg) daily for 5 days. Last time this was given: 20 mg on June 23, 2026 7:36 AM Start taking on: June 24, 2026 What changed: See the new instructions.	Wait until Jun 24				
sulfamethoxazole-trimethoprim 800-160 MG per tablet Commonly known as: BACTRIM DS Take 1 tablet by mouth three times a week for 70 doses Indications: COPD PPX Last time this was given: 1 tablet on June 22, 2026 9:45 AM Start taking on: June 24, 2026	Wait until Jun 24				
theophylline 300 MG SR capsule Commonly known as: THEO-24 Take 1 capsule (300 mg) by mouth daily Last time this was given: 300 mg on June 23, 2026 7:34 AM	 1 capsule				

 * This list has 2 medication(s) that are the same as other medications prescribed for you. Read the directions carefully, and ask your doctor or other care provider to review them with you.

Where to pick up your medications



Pick up these medications at Omnicare of Henderson - Henderson, KY - 1900 Highway 41 North

acetaminophen • albuterol (2 prescriptions) • aspirin • atorvaSTATin • benzonatate • budesonide • busPIRone • DSS • Dupixent • ferrous sulfate • FLUoxetine • furosemide • guaifENesin • ipratropium-albuterol • lisinopril • metFORMIN • metoprolol succinate • morphine • omeprazole • ondansetron • Ozempic (0.25 or 0.5 MG/DOSE) • predniSONE • sulfamethoxazole-trimethoprim

Address: 1900 Highway 41 North Unit 4, Henderson KY 42420
Phone: 800-426-8708

Patient Valuables/Belongings Reported

Patient Valuables/Belongings

Information provided by: patient

Relationship: self

Patient Valuables/Belongings Sent Home -Encourage Patient to Send Valuables Home with Family/Caregiver

Valuables/Belongings Sent Home: None

Patient Belongings Keeping at Bedside

Belongings at Bedside: Clothing, Glasses, Cell Phone, Jewelry, Other (comment)

All Jewelry Removed?: No (comment)

Ensure all piercings removed: Other (comment)

Patient Valuables Sent for Safekeeping - Place Medication in Separate Valuables Bag

Valuables Sent for Safekeeping?: None

Dyspnea

WHAT YOU NEED TO KNOW:

What is dyspnea? Dyspnea is breathing difficulty or discomfort. You may have labored, painful, or shallow breathing. You may feel breathless or short of breath. Dyspnea can occur during rest or with activity. You may have dyspnea for a short time, or it might become chronic. Dyspnea is often a symptom of a disease or condition.

What signs and symptoms can occur with dyspnea?

- Chest tightness or pain
- A cough, or a coarse or high-pitched noise when you breathe
- Pale and sweaty, cool skin
- Confusion and tiredness
- Bluish-gray lips or nails

What increases my risk for dyspnea?

- Swelling in the throat from an infection or allergic reaction
- Lung conditions such as asthma, COPD, cancer, infection, or a blood clot
- Heart conditions such as abnormal heartbeats, heart failure, or coronary artery disease
- Smoking, exposure to chemicals such as carbon monoxide, or too much aspirin
- A condition that affects your central nervous system, such as a spinal cord injury or nerve damage
- Enlarged abdomen because you are overweight, pregnant, or have ascites (fluid in the abdomen) from liver disease
- Anemia (low red blood cell count), anxiety, panic, or going to a high altitude

How is the cause of dyspnea diagnosed? Your healthcare provider may ask when your dyspnea began and what you were doing. Tell him or her how often you have dyspnea and what makes it worse or better. Tell your healthcare provider about medicines you take. Describe any other symptoms, such as pain or a fever. Your healthcare provider will listen to you breathe and watch for irregular breathing. You may also need the following:

- **A pulse oximeter** is a device that measures the amount of oxygen in your blood. A cord with a clip or sticky strip is placed on your finger, ear, or toe. The other end of the cord is hooked to a machine.
- **Blood tests** may show your healthcare provider if you are at risk for blood clots or heart failure. Blood tests can also show if you have anemia or an infection.
- **X-ray** pictures may show signs of infection or fluid around your heart or lungs.
- **Exercise tests** help your healthcare provider learn if you have symptoms, along with dyspnea, that limit activity. Symptoms include leg pain, fatigue, and weakness. Exercise tests can also show if your dyspnea is caused by heart problems.
- **CT scan** pictures may show blood clots or an area of disease in your lungs. You may be given contrast liquid to help your lungs show up better in the pictures. Tell the healthcare provider if you have ever had an allergic reaction to contrast liquid.
- **An echocardiogram** is a type of ultrasound. Sound waves are used to show the structure and function of your heart.
- **An EKG** is a test that measures the electrical activity of your heart.

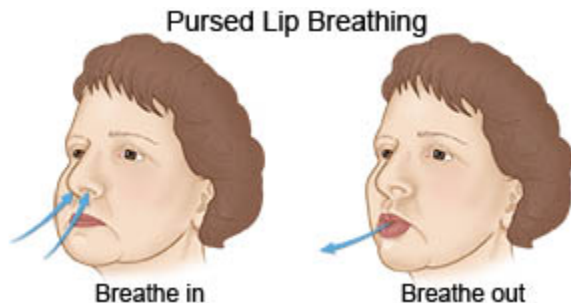
How is dyspnea treated? You may need treatment if your symptoms prevent you from doing your daily activities. The condition causing your dyspnea may need to be treated. You may also need the following to improve your symptoms:

- **Oxygen therapy** may be used to help you breathe easier. You may need oxygen if your blood oxygen level is lower than it should be.
- **Medicines** may be used to treat the cause of your dyspnea. Medicines may reduce swelling in your airway or decrease extra fluid from around your heart or lungs. Other medicines may be used to decrease anxiety and help you feel calm and relaxed.

- **Pulmonary rehabilitation** is used to reduce your symptoms while keeping you active. You may learn breathing techniques, muscle strengthening, and how to pace yourself when you are active.

How can I manage long-term dyspnea?

- **Create an action plan.** You and your healthcare provider can work together to create a plan for how to handle episodes of dyspnea. The plan can include daily activities, treatment changes, and what to do if you have severe breathing problems.
- **Lean forward on your elbows when you sit.** This helps your lungs expand and may make it easier to breathe.
- **Use pursed-lip breathing any time you feel short of breath.** Breathe in through your nose and then slowly breathe out through your mouth with your lips slightly puckered. It should take you twice as long to breathe out as it did to breathe in.



- **Do not smoke.** Nicotine and other chemicals in cigarettes and cigars can cause lung damage and make it harder to breathe. Ask your healthcare provider for information if you currently smoke and need help to quit. E-cigarettes or smokeless tobacco still contain nicotine. Talk to your healthcare provider before you use these products.
- **Reach or maintain a healthy weight.** Your healthcare provider can help you create a safe weight loss plan if you are overweight.
- **Exercise as directed.** Exercise can help your lungs work more easily. Exercise can also help you lose weight if needed. Try to get at least 30 minutes of exercise most days of the week. Your healthcare provider can help you create an exercise plan that is safe for you.

When should I seek immediate care?

- Your signs and symptoms are the same or worse within 24 hours of treatment.

- You have shaking chills or a fever over 102°F.
- You have new pain, pressure, or tightness in your chest.
- You have a new or worse cough or wheezing, or you cough up blood.
- You feel like you cannot get enough air.
- The skin over your ribs or on your neck sinks in when you breathe.
- You have a severe headache with vomiting and abdominal pain.
- You feel confused or dizzy.

When should I call my doctor or specialist?

- You have questions or concerns about your condition or care.

CARE AGREEMENT:

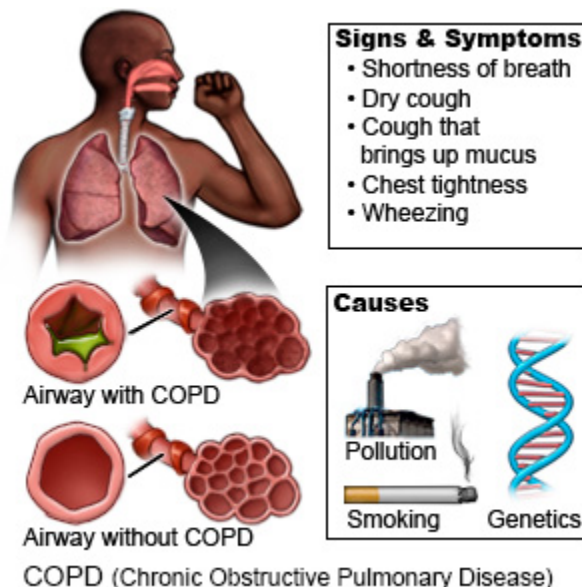
You have the right to help plan your care. Learn about your health condition and how it may be treated. Discuss treatment options with your healthcare providers to decide what care you want to receive. You always have the right to refuse treatment. The above information is an educational aid only. It is not intended as medical advice for individual conditions or treatments. Talk to your doctor, nurse or pharmacist before following any medical regimen to see if it is safe and effective for you.

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COPD (Chronic Obstructive Pulmonary Disease)

WHAT YOU NEED TO KNOW:

COPD (chronic obstructive pulmonary disease) can get worse quickly. Your healthcare providers will help you create a care plan to use at home. The plan will give directions on how to prevent or manage shortness of breath. Your family members or anyone who cares for you will also get directions to help you.



DISCHARGE INSTRUCTIONS:

Call your local emergency number (911 in the US) if:

- You feel lightheaded, short of breath, and have chest pain.

Seek care immediately if:

- You cough up blood.
- You are confused, dizzy, or feel faint.
- Your arm or leg feels warm, tender, and painful. It may look swollen and red.

Call your doctor if:

- You have increased shortness of breath.

- You need more medicine than usual to control your symptoms.
- You are coughing or wheezing more than usual.
- You are coughing up more mucus, or it has a new color or odor.
- You gain more than 3 pounds in a week.
- You have a fever, a runny or stuffy nose, and a sore throat, or other cold or flu symptoms.
- Your skin, lips, or nails start to turn blue.
- You have swelling in your legs or ankles.
- You are very tired or weak for more than a day.
- You notice changes in your mood, or changes in your ability to think or concentrate.
- You have questions or concerns about your condition or care.

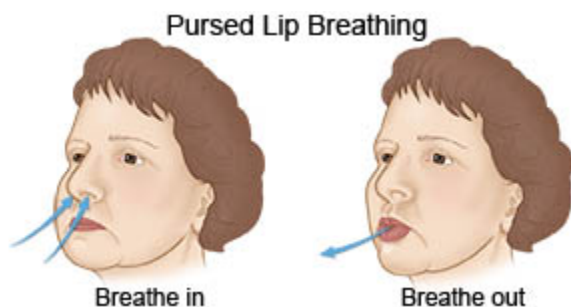
Medicines:

- **Short-acting bronchodilators** may be called rescue inhalers or relievers. They relieve sudden, severe symptoms and start to work right away.
- **Long-acting bronchodilators** may be called controllers or maintenance medicine. This medicine helps open the airway over time, and is used to decrease and prevent breathing problems. Long-acting bronchodilators should not be used to treat sudden, severe symptoms, such as trouble breathing.
- **Antibiotics** may be given for up to 5 days to treat a bacterial infection during an exacerbation.
- **Take your medicine as directed.** Contact your healthcare provider if you think your medicine is not helping or if you have side effects. Tell your provider if you are allergic to any medicine. Keep a list of the medicines, vitamins,

and herbs you take. Include the amounts, and when and why you take them. Bring the list or the pill bottles to follow-up visits. Carry your medicine list with you in case of an emergency.

Help make breathing easier:

- **Use pursed-lip breathing any time you feel short of breath.** Take a deep breath in through your nose. Slowly breathe out through your mouth with your lips pursed. Try to take 2 times as long to breathe out as to breathe in. This helps you get rid of as much air from your lungs as possible. You can also practice this breathing pattern while you bend, lift, climb stairs, or exercise. It slows down your breathing and helps move more air in and out of your lungs.



- **Avoid anything that makes your symptoms worse.** Stay out of high altitudes and places with high humidity. Stay inside, or cover your mouth and nose with a scarf when you are outside in cold weather. Stay inside on days when air pollution or pollen counts are high. Do not use aerosol sprays such as deodorant, bug spray, and hairspray.
- **Exercise as directed.** Your healthcare provider may recommend at least 20 minutes of exercise each day to help increase your energy and decrease shortness of breath. Talk to your provider about the best exercise plan for you.



Walk Together

Manage COPD and help prevent exacerbations: COPD is a serious condition that gets worse over time. A COPD exacerbation means your symptoms suddenly get worse. It is important to prevent exacerbations. An exacerbation can cause more lung damage. COPD cannot be cured, but you can take action to feel better and prevent exacerbations:

- **Do not smoke.** Nicotine and other chemicals in cigarettes and cigars can cause lung damage and make your COPD worse. Ask your healthcare provider for information if you currently smoke and need help to quit. E-cigarettes or smokeless tobacco still contain nicotine. Talk to your provider before you use these products.
- **Avoid secondhand smoke.** This is smoke another person exhales. Even if you have never smoked or have quit, it is important to avoid secondhand smoke. This smoke can also cause lung damage or trigger an exacerbation.
- **Go to pulmonary rehabilitation (rehab) if directed.** Rehab is a program run by specialists who help you learn to manage COPD. Examples include a pulmonologist (lung specialist), dietitian, or exercise therapist. The specialists will help you make a plan to avoid triggers that cause an exacerbation.
- **Take your medicines as directed.** Refill your medicines before you are out so that you do not miss a dose. Ask your provider if you have any questions on how to take your medicines.
- **Protect yourself from germs.** Germs can get into your lungs and cause an infection. An infection in your lungs can create more mucus and make it harder to breathe. An infection can also create swelling in your airway and prevent air from getting in. You can decrease your risk for infection by doing the following:



- **Wash your hands often.** Wash your hands several times each day. Wash after you use the bathroom, change a child's diaper, and before you prepare or eat food. Wash your hands for at least 20 seconds. Rinse with warm, running water. Then dry your hands with a clean towel or paper towel. Use hand sanitizer that contains alcohol if soap and water are not available.

Handwashing



- **Always cover your mouth when you cough.** Cough into a tissue or your shirtsleeve so you do not spread germs from your hands.
- **Try to avoid people who have a cold or the flu.** If you are sick, stay away from others as much as possible.
- **Ask about vaccines you may need.** Your provider may recommend these and other vaccines:
 - **Ask your healthcare provider about the flu and pneumonia vaccines.** All adults should get the flu (influenza) vaccine as soon as recommended each year, usually in September or October. The pneumonia vaccine is recommended for all adults aged 50 or older to prevent pneumococcal disease, such as pneumonia. Adults aged 19 to 49 years who are at high risk for pneumococcal disease should also receive the vaccine. You may need 1 dose or 2. The number depends on the vaccine used and your risk

factors.

- **COVID-19 vaccines are given to adults as a shot.** At least 1 dose of an updated vaccine is recommended for all adults. COVID-19 vaccines are updated throughout the year. Adults 65 or older need a second dose of updated vaccine at least 4 months after the first dose. Your healthcare provider can help you schedule all needed doses as updated vaccines become available.
- **Drink liquids as directed.** You may need to drink more liquid than usual. Liquid will help to keep your air passages moist and help you cough up mucus. Ask how much liquid to drink each day and which liquids are best for you.

Follow up with your doctor as directed: You may need more tests. Your doctor may refer you to a specialist, depending on your needs. Some specialist services may be available through your pulmonary rehab program. Write down your questions so you remember to ask them during your visits.

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The above information is an educational aid only. It is not intended as medical advice for individual conditions or treatments. Talk to your doctor, nurse or pharmacist before following any medical regimen to see if it is safe and effective for you.



**Schedule your next appointment
online at deaconess.com/schedulenow**
(or scan this code with your phone or tablet)

DHS IP PHYSICIAN AUTHORIZATION (From admission, onward)

Start		Ordered
06/23/26 0000	Physician/Nurse Practitioner Authorization Order Creator: Malik, Ahmad T, MD Signature: Electronically signed by Ahmad T Malik, MD	06/23/26 1515

Additional Information

Patients can request an estimate of the costs for nonemergency health care services from their health care providers and the health facility. For an estimate, please contact 812-450-2678(COST) or email deaconesscostquote@deaconess.com.